Table 2 Themes, subthemes, and illustrative quotes

SEM level: Themes	Subthemes	Illustration
Intrapersonal: living with dementia and caregiving	Living with dementia – being forgetful and dependent	"... So she could say where she felt pain when she was in pain right at that moment. But she forgot afterwards… That was the problem. That's the problem with old dementia patients." (FCG, p326)[47]
		"I myself get upset when I misplace things, which makes them [her family members] search a lot to find them..." (OIWD, p3)[19]
		"So she went to do something that she was doing all her life and doesn't remember what [ingredient] she has to put in next. …" (FCG, p165)[48]
		"I call someone to ask them to bring something to me. When they come, I have forgotten what I wanted. Then I just greet and thank them, without saying I forgot what I wanted." (OIWD, p4)[19]
		"...I get embarrassed when they are not able to find things. I regret that I took those things. But, you know, I cannot stop myself organizing my surroundings." (PWD, p5)[19]
		"… You have your own place, you have money. Why should you worry then... you know here, I can go out whenever I want." (OIWD, p5)[19]
		"It makes me feel how far am I going, what is the worst that can happen, you know it make me feel inadequate . . . sometimes I think oh Lord what is going on, am I getting off my brain or what?" (OIWD, p44)[31]
		"I would rather be dead than drive somebody least of all my own wife to that sort of situation." (OIWD, p45)[31]
		"Because before I used to do everything for myself, go out, go to help people and do different things you know, around. I did everything for myself. But now I can't do anything, I always forgetting you see." (OIWD, p46)[31]
		"… It's a really bad disease to have, not knowing who you are, what time it is or where you are, anything ... It was degrading. We didn't know it would get that bad, and we didn't know that it was so degrading for the person." (FCG, p389)[50]
	Caregiving – emotionally and physically challenging	"I have to be at her side all the time, going to the bathroom, etc. How could I have time to take care of myself? Making a doctor's appointment for my wife becomes my first priority, but my health? I don't have time to think about it." (FCG, p38)[37]
		"... My back becomes extremely sore after all that." (FCG, p293)[49]
		''I was pretty healthy, but actually after starting taking care of patients with dementia, I got sick due to stress.'' Lee et al. (HCW, p429)[38]
		"Despite of my full-time job and voluntary work, I was always there for my father... I kept going, I wanted to do everything for my father... The others [non-caregiving siblings] wouldn't do that." (FCG, p344)[35]
		"She [caregiver's daughter] is busy most of the time, so I don't go to her for little stuff . . . and ask her help only for really important things." (FCG, p39)[37]
		"… It's not about me, but about her... I expected them to do the same as I'm doing." (FCG, p344)[35]
		"There were times that I screamed [at her]: "You ruined my life!" But it's my mother, and later when I look at her and see a person who's half dead… I think: "Why do I say those things? But it happens when I'm tired." (FCG, p346)[35]

		"I took three years off because of my father […] and I haven't gone anywhere […] it wasn't possible with my father." (FCG, p870)[39]
		"Until January he was working and now we closed our shop because we have to watch my mother." (FCG, p870)[39]
		"... But I have other things (i.e., responsibilities) in my life too, so if I only endure, it is not fair to my kids… I have had to accept a lot of criticism, bad criticism." (FCG, p24)[16]
		"My circle of friends is getting smaller and smaller. I can't make any time for myself to go outside for a bit." (FCG, p79)[33]
		"I don't have time for myself or my children or my home." (FCG, p869)[39]
		"I didn't know much details about the disease, so I had a very difficult time when she asked me the same question over and over—like fifteen times. … We need to know more about the disease so we can be more patient while dealing with them." (HCW, p431)[38]
		"What if she outlives me? Her life will become even more miserable. Yes, she's got our children to look after her, but it's different to the care from a husband." (FCG, p295)[49]
		"I don't know whether there are any supportive services available in the community and nobody told us." (FCG, p129)[46]
		"… But we do not have mechanisms for how to manage stress, to handle all these tasks—and the migration process has in many ways isolated us…" (FCG, p21)[16]
		"... I can't decide how I want to care for my mother because they consider it a bad choice. If I share the care for my mother with a professional they'll tell me I'm a bad daughter for not caring for my mother after all she has done for me." (FCG, p78)[33]
	Caregiving – obligation and benefits	"That's how it is in our culture. It is not forced. It is love I got from him when I was small. Now I am returning what I got. It is my responsibility. He has only me." (FCG, p6)[32]
		"I did it for Allah. It's for my religion. I don't expect anything [in return]…" (FCG, p345)[35]
		"I feel very happy taking care of her because she was a good mother and she deserves to get back a little of that—of everything she did for us." (FCG, p168)[48]
		"This is our mother, this is our father, we have to watch them. They have done so much for us, so we have to take care of them now." (FCG, p868)[39]
		"… Sending him to a nursing home made me feel like I was abandoning him because I didn't care about him anymore and that made me feel like I was an unfilial daughter, which broke my heart." (FCG, p323)[47]
		"When we grew up, we saw my mother dealing with my grandmother. So we just sort of pass it on." (FCG, 166)[48]
		"She [caregiver's daughter] is busy most of the time, so I don't go to her for little stuff . . . and ask her help only for really important things." (FCG, p39)[37]
		"… I really, really don't like it at all. I don't know how long I can take care of her [mother-in-law], but as long as I can, I will do it, so I do not ask any help from other family members." (FCG, p39)[37]

		"It is very fulfilling for me - I'm pleased to be with her and pleased that I've got her, and so I'm fully committed to it. It's fulfilling for me, and yes it's very tough. ...I get a lot out of it." (FCG, p79)[34]
		"I get a lot of satisfaction from it because I think that there's nothing better in the whole world. . . I hope I'm doing it right and well, when I hear him talking about me and about the care I'm giving, that is very fulfilling." (FCG, p78)[34]
	Lack of knowledge about dementia	"I think that people often don't know that their relatives suffer from dementia. they can't read the symptoms. Perhaps they don't know about illnesses such as dementia." (HCW, p4)[32]
		"I didn't know much details about the disease, so I had a very difficult time when she asked me the same question over and over – like fifteen times… We need to know more about the disease so we can be more patient while dealing with them." (HCW, p431)[38]
		'I would say it might be that we were too slow in getting help for her and that it took a while for us to understand that she had dementia. I think it would have been different if we had more knowledge. It was after she burnt something at home that we decided that was enough and that we had to find out what was wrong'. (FCG, p5)[32]
		'It was by chance. I was talking to my physiotherapist…he asked me if I knew about night care, and gave me the name of the organization. … I wouldn't know about it if he hadn't told me'. (FCG, p9)[44]
Interpersonal: family relationship	Family conflicts	"They would argue like ''why don't you take care, why only I have to do it?'' If it's just one or two years, it's okay, but when it becomes like 5, 6, 7 years, then conflicts occur within the family...Like the old saying ''there is not piety for a lingering illness.'' When the disease becomes prolonged, the conflicts get worse." (HCW, p431)[38]
		"Look, you only see him for two hours; you don't know what happens the other hours." (FCG, p347)[35]
		"We are left only with two of us, me and my sister K – my other sisters and brother don't help anymore." (FCG, p4)[43]
	Helping the PWD	"I say to her ''this dementia is not working well for you'' and I kind of make it a joke for her to have a laugh." (FCG, p293)[49]
		"When I used to see her repeating a lot without making sense I would grab her hand and pray for her … she would feel better, emotionally." (FCG, p293)[49]
		"It's hard to say how much they understand, it is clearly visible from their faces and their expressions and so... how they react..." (HCW, p319)[45]
		"... I notice that she becomes very happy when I go to her... for us, she has a lot to tell, when nobody else is in there..." (HCW, p320)[45]
Organizational: barriers to dementia care	Service barriers	"People with dementia – I don't think they have any services. There is no access and if they have tried using them, then they are not appropriate to their needs." (HCW, p394)[50]
		"Immigrants know very little about how to help an elderly person with dementia in Norway. I must tell you that I have lived here for 22 years and there is a problem with it, because we don't know where to get information and help." (FCG, p5)[32]
		"People often lack sufficient information. You get everything in Norwegian. When you go to doctor, everything is in Norwegian. When you go to other places, everything is in Norwegian." (HCW, p8)[32]

		"There are low-income housing apartments for older Chinese adults, but there are no special housing facilities for Chinese dementia patients (like my husband)." (FCG, p129)[46]
		"If you send them to a proper good day center [i.e., a mainstream service], nothing is available for them. They don't understand their religious needs or anything … They should have water in the loo, you know, because they don't use tissues, and then, if they have a home help, they need a male home help in the house not a female, if he is living on his own." (HCW, p392)[50]
		"Well, I think it is important, so that they can identify themselves with it […] that's why it is important that there are services that consider the culture or language. So one feels more understood, also as a family member." (FCG, p871)[39]
		"For us, you see, it is normal to wash someone in a dry way, just using a toilet glove, but he (patient with dementia) started to get agitated. We didn't know what to do. We finally called the intercultural mediator, who explained to us that he needed to be wet washed..." (HCW, p6)[43]
	Language barriers	"We wanted to look for a home taker [formal caregiver] for my mother, but we could not find bilingual and skilled formal caretakers, and there are few residential care facilities for Chinese patients." (FCG, p129)[46]
		"Because we don't speak English, we can't take advantage of available services… When I look for services, they are available, but I can't use them even though I know that they are there… I can't take her [care recipient] there [nursing home] because she doesn't speak English, and she can't live eating American food." (FCG, p41)[37]
		"… same language would be important, although my grandmother can speak German […] but not in a way that she can express emotions…" (FCG, p871)[39]
		"People often lack sufficient information. You get everything in Norwegian. When you go to doctor, everything is in Norwegian. When you go to other places, everything is in Norwegian." (HCW, p8)[32]
	Hesitance toward formal care	"They leave people there, suffering. They have no contact at all with the patients. It's just their job and that's it - just waiting for them to kick the bucket…" (FCG, p77)[34]
		"I felt that the nursing home did not treat dementia at all, and I think the nursing homes are not the place for providing the treatment or the prescription for improvement of dementia." (FCG, p325)[47]
		"It's part of the culture, isn't it? A kind of taboo... I mean, putting your father in a care home is a big no-no for us." (FCG, p77)[34]
		"… If the life of the ill person is in danger or if their illness means that they are harming others and if we cannot resolve the situation ourselves and if it really cannot go on like that anymore, then a care home could be an option…" (FCG, p77)[34]
Community: stigma and discrimination	Hiding dementia	"We do not want to tell the neighbors or the friends in church, first, it is not an honorable thing to share and, second, people will look at mother and us differently when they hear it." (FCG, p130)[46]
		"The problem is that they don't wish to admit or accept that their loved one is demented." (HCW, p8)[36]
		"These relatives carry a sense of shame about their parent's dementia." (HCW, p8)[36]

		"It is kept closed. People don't want to reveal that it is in their families. It may bring problems when marriages are being arranged." (HCW, p392)[50]
	Shame and discrimination	"I'm very careful about what I say (to the family members), to avoid being blamed for discrimination." (HCW, p6)[43]
		"They know you're Spanish, and I'm telling you, they do discriminate, especially if they hear you on the phone and they know you have an accent." (FCG, p167)[48]
		"She (family member) was totally exhausted, had responsibility for her own children, to follow them to school and kindergarten. Her GP advised her to apply for help from the home-based services, but she did not want to receive such help. She worried about what people around her would think about that." (FCG, p21)[16]
		"When Korean people hear that somebody sent their parents to a nursing home, they point finger at (look down on) them.'' (HCW, p431)[38]
		"You should have seen the way how he (a doctor) talked to me and to my father. Without any respect, like we were animals... it hurts me that my father has to endure such racism at this stage of his life." (FCG, p6)[43]
		"My daughter and son-in-law used to take us out to eat, but ever since my husband had dementia [at very early stage], neither my husband nor I are willing to eat outside." (FCG, p130)[46]
Policy: legal and financial issues	Legal issues	"You see, we could call intercultural care mediators or translation services when we had a patient with a different background – that was helpful, but due to hospital decisions we cannot use these services anymore." (HCW, p6)[43]
	Financial issues	"There is something called omsorgslønn [care allowance] but it is very low pay. The municipality pays for this care allowance. The municipality always wants people to use the services from the municipality first. If that is not enough, then they say, 'We can pay a little bit of care allowance." (HCG, p9)[32]